

Common Skin Diseases for Management or Referral at Primary Health Care Level

WHO guidance with ICD-11 Mortality and Morbidity Statistics (MMS) codes, release
2026-01

Version 1.0

For external expert review draft

Disclaimer

This document has been prepared to support the recognition, management and referral of common skin diseases at the primary health care (PHC) level. It is intended to assist Ministries of Health, programme managers, educators and primary health care providers in strengthening integrated skin health services.

The classification supports clinical decision-making, training, service delivery and health information systems. It should be adapted to national epidemiology, available resources and existing clinical guidelines.

This document does not replace national clinical guidelines. Countries should adapt its recommendations in accordance with national policies, local epidemiology, available resources and health system capacity.

Relationship to the GAP-SKIN companion implementation package. This classification is the PHC clinical-service anchor within the GAP-SKIN companion implementation package. GAP-SKIN defines the strategic direction; this document supports recognition, initial management, referral, coding, training and service delivery. It should be used alongside national clinical guidelines, the Two-Phase Prioritization Framework, the Training and Competency Framework, NAP-Skin Guidance and the Indicator Compendium. It does not rank diseases for national priority-setting or create separate reporting requirements.

Artificial intelligence (AI) was used to support editorial tasks, including language editing, formatting and consistency checks. All scientific content, clinical recommendations and technical information have been reviewed and validated by subject matter experts. Final responsibility for the content rests with the authors and expert reviewers.

DRAFT

Preface

Skin diseases are among the most common reasons for seeking health care and are encountered daily in primary health care settings worldwide. Many conditions can be recognised and managed effectively at the primary care level, while others require timely referral to prevent complications, disability or unnecessary morbidity.

This publication was developed to provide a practical classification of common skin diseases that supports clinical decision-making, standardised use of ICD-11 Mortality and Morbidity Statistics (MMS) terminology, and integration of skin health into primary health care services. The guidance is designed for use by Ministries of Health, programme managers and primary health care providers and is intended to complement national clinical guidelines.

The emphasis throughout this publication is on early recognition, appropriate initial management and timely referral. Recommendations have been written to be practical, evidence-informed and adaptable to different health system contexts while maintaining consistency with international standards.

This publication reflects a collaborative technical development process involving subject matter experts, technical editing and evidence review. The final clinical content has been reviewed and validated by expert reviewers before publication.

Acknowledgements

The authors gratefully acknowledge the contributions of dermatologists, primary health care professionals, technical reviewers and programme experts who provided valuable comments during the development and review of this publication. Their expertise helped strengthen the scientific accuracy, clinical relevance and practical applicability of this guidance.

The authors also acknowledge the use of artificial intelligence (AI)-assisted editorial support during manuscript preparation. AI was used exclusively to improve language, consistency, formatting and editorial quality. All scientific content, clinical recommendations and final editorial decisions were reviewed and validated by the authors and expert reviewers before publication.

DRAFT

Contents

- 1. 1. Introduction**
 - 2. 2. Scope and purpose**
 - 3. 3. Intended users**
 - 4. 4. How to use this classification**
 - 5. 5. Primary health care management categories**
 - 6. 6. Classification of common skin diseases**
 - 6.1 Infectious skin diseases
 - 6.2 Skin neglected tropical diseases
 - 6.3 Inflammatory and chronic dermatoses
 - 6.4 Pigmentary disorders
 - 6.5 Chronic wounds, ulcers and lymphatic disorders
 - 6.6 Other common PHC presentations
 - 6.7 Benign skin lesions
 - 6.8 Pre-cancerous and malignant skin conditions
 - 6.9 Systemic, drug-related and severe skin conditions
 - 7. 7. Methodological note**
 - 8. 8. References**
 - 9. 9. Annexes**
-

Introduction

Skin diseases are among the most common health conditions worldwide and contribute substantially to morbidity, disability, stigma and reduced quality of life. Many common skin diseases can be accurately recognised and effectively managed at the primary health care (PHC) level using simple clinical assessment and evidence-based interventions.

Strengthening integrated skin health within PHC improves access to timely diagnosis, appropriate treatment and referral, while supporting universal health coverage and person-centred care. A standardised approach also promotes efficient use of health resources, improves surveillance and facilitates integration with national health programmes.

This document supports the WHO Integrated Skin Health approach and aligns with World Health Assembly resolution WHA78.15, which recognises skin diseases as a global public health priority. It is also consistent with the forthcoming Global Action Plan on Skin Diseases (2026–2035) and complements integrated approaches to skin-related neglected tropical diseases (skin NTDs).

The classification provides a practical framework to support the recognition, management and referral of common skin diseases at the PHC level. It combines standardised disease classifications with ICD-11 Mortality and Morbidity Statistics (MMS) codes and concise clinical guidance to support routine service delivery, training and health information systems.

Scope and purpose

This document provides a practical classification of common skin diseases that can be recognised, managed or referred at the primary health care (PHC) level.

The classification is intended to:

- support clinical decision-making at PHC level;
- strengthen integrated skin health services;
- facilitate implementation of national training programmes;
- promote standardised use of ICD-11 MMS codes;
- improve disease surveillance and reporting; and
- support planning, monitoring and evaluation of skin health programmes.

The classification is not exhaustive and should be adapted to national disease epidemiology, health priorities and available resources.

Important distinction. This classification should not be interpreted as a global ranking of disease importance. It is a practical PHC decision-support and service-delivery tool. National disease or service priorities should be determined through country situation analysis and the GAP-SKIN Two-Phase Prioritization Framework.

Intended users

This document is intended for:

- Ministries of Health;
 - primary health care providers;
 - programme managers;
 - dermatology services;
 - educators and training institutions;
 - public health practitioners; and
 - partners supporting implementation of integrated skin health programmes.
-

How to use this classification

Each disease has been assigned a PHC management category indicating whether it can generally be managed at PHC level, should be recognised and referred according to local capacity, or requires urgent referral.

Each disease entry includes:

- the disease name;
- ICD-11 Mortality and Morbidity Statistics (MMS) code(s);
- key clinical features;
- recommended PHC management; and

- • referral considerations, where appropriate.

Countries are encouraged to adapt the [scheme](#) to [accommodate](#) national clinical guidelines, local epidemiology and available resources.

DRAFT

Use with other companion documents. The PHC management categories provide the clinical and service-delivery anchor for competency-based training, service-package design, referral pathways, coding, data recording and quality improvement. Countries should connect the categories to NAP-Skin priorities, the Training and Competency Framework and the GAP-SKIN Indicator Compendium.

Primary Health Care Management Categories

This classification uses three management categories to guide decision-making at the primary health care (PHC) level. The categories indicate whether a condition can generally be managed at PHC level, should be recognised and referred according to local capacity, or requires urgent referral.

The management category assigned to each condition should always be interpreted in the context of national clinical guidelines, local epidemiology, available resources and health system capacity. Individual patient factors, including age, pregnancy, immunosuppression and the presence of severe disease or complications, may also influence management decisions.

PHC category	Definition	Expected action at PHC level
Manage at PHC level	The condition can generally be recognized, diagnosed and managed at the primary health care level using standard clinical assessment and locally available treatments.	Provide appropriate treatment, counselling and follow-up. Refer if there is diagnostic uncertainty, treatment failure or complications.
Recognise and consider referral	The condition should be recognized at PHC level. Initial assessment and basic management may be appropriate; however, referral may be required depending on disease severity, available expertise or local service capacity.	Provide initial assessment and supportive care where appropriate, and refer according to national guidance and local referral pathways.
Urgent referral	The condition requires urgent referral because of the risk of severe morbidity, disability or death, or because prompt specialist assessment and treatment are required.	Stabilize the patient where appropriate, initiate emergency management if indicated, and arrange urgent referral according to locally defined urgent referral pathways.

Note: Some conditions are designated as context-dependent. This indicates that management may differ according to local epidemiology, available resources and health system capacity. Where appropriate expertise and resources are available, these conditions may be managed at PHC level; otherwise they should be recognised promptly and referred in accordance with national clinical guidelines [National adaptation should document any category changes and preserve patient safety, equity and continuity of care.](#)

Principles for Primary Health Care Management

The management recommendations in this classification are intended to support decision-making at the primary health care (PHC) level. They are based on the principles of integrated, person-centred care and should be implemented in accordance with national clinical guidelines and local health system capacity.

PHC providers should:

- recognise common skin diseases through clinical assessment and appropriate history taking;
- identify signs of severe disease, complications or systemic illness that require referral;
- initiate evidence-based treatment using locally available medicines and supplies;
- provide patient education on treatment adherence, skin care and prevention of disease transmission where appropriate;
- identify and manage underlying risk factors and comorbidities where feasible; and
- arrange timely referral when specialist assessment or higher levels of care are required.

Equity and stigma-responsive care. PHC providers should recognize that visible skin conditions may be associated with stigma, discrimination, delayed care-seeking and social exclusion. Respectful communication, privacy, non-discrimination, disability inclusion, attention to presentation across different skin tones and timely referral are essential components of quality PHC skin health services.

Clinical judgement should always be applied when assessing individual patients. Management decisions may vary according to age, pregnancy, immune status, comorbidities, disease severity and local resource availability

6. Classification of Common Skin Diseases

This classification presents common skin diseases that can be recognised, managed or referred at the primary health care (PHC) level. The diseases are grouped according to their predominant clinical characteristics to facilitate diagnosis, management and referral in routine clinical practice.

For each condition, the classification provides:

- the PHC management category;
- the disease name;
- the corresponding ICD-11 Mortality and Morbidity Statistics (MMS) code(s);
- key clinical features to support recognition;
- recommended PHC management; and
- referral criteria, where appropriate.

The classification is intended to support clinical decision-making and should be used in conjunction with national clinical guidelines, local referral pathways and available resources.

ICD-11 coding quality assurance. Codes and titles in this draft were checked against the WHO ICD-11 MMS release 2026-01. Where a diagnosis requires selection of a subtype, site, cause, histopathology or extension code, the table identifies the parent category or states that postcoordination or cluster coding is required. National implementation guidance may require additional coding detail.

6.1 Infectious skin diseases

Infectious skin diseases are among the most common conditions encountered in primary health care. They are caused by bacteria, fungi, viruses and parasites and account for a substantial proportion of consultations in many settings.

Most infectious skin diseases can be diagnosed clinically and managed effectively at the PHC level using simple, evidence-based interventions. Early recognition and appropriate treatment reduce morbidity, prevent complications and limit transmission within households and communities.

PHC providers should be able to recognise common infectious skin diseases, initiate appropriate management and identify patients who require referral for specialist assessment or hospital-based care.

6.1.1 Bacterial skin infections

Bacterial skin infections are among the most common conditions encountered at the primary health care (PHC) level. Most uncomplicated infections can be recognised clinically and managed effectively using evidence-informed interventions. Early diagnosis and appropriate management reduce complications, prevent disease progression and limit transmission where applicable.

Table 6.1. Bacterial skin infections

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	Recommended PHC management	Referral criteria
Manage at PHC level (core)	Impetigo (superficial bacterial infection, pyoderma)	1B72.0 Bullous impetigo; 1B72.1 Non-bullous impetigo	• Highly contagious superficial bacterial infection, usually caused by <i>Staphylococcus aureus</i> or <i>Streptococcus pyogenes</i> .	• Treat according to national clinical guidelines using topical or oral antibiotics, depending on disease severity and extent.	Refer if: • disease is extensive; • systemic illness is present;

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	Recommended PHC management	Referral criteria
			- Honey-coloured crusts or small blisters on an erythematous base, most commonly on the face and extremities. - Common in children.	- Advise on hand hygiene, wound care and avoidance of sharing personal items. - Counsel patients and caregivers on preventing transmission.	- recurrent infection occurs; - treatment fails; or - complications are suspected.
Manage at PHC level (core)	Cellulitis and erysipelas	1B70.Z Cellulitis due to unspecified bacterium; use a more specific code only when the causative organism is documented	- Acute bacterial infection of the skin and subcutaneous tissues. - Erysipelas presents with a well-demarcated, raised erythematous plaque, often accompanied by fever. - Cellulitis presents with diffuse erythema, warmth, tenderness and swelling, usually with poorly defined margins.	- Initiate systemic antibiotic therapy according to national clinical guidelines. - Elevate the affected limb where appropriate. - Identify and manage potential portals of entry, including wounds and tinea pedis. - Monitor for clinical progression.	Urgently refer if: - infection progresses rapidly; - systemic toxicity develops; - necrotizing soft-tissue infection is suspected; - an abscess requires surgical management; or - there is no response to treatment after 48 hours.
Manage at PHC level (core)	Folliculitis and skin abscess	1B74 Superficial bacterial folliculitis; 1B75.3 Pyogenic skin abscess	- Folliculitis presents as pustules centred on hair follicles. - Skin abscess presents as a painful, fluctuant collection of pus with surrounding erythema. <i>Staphylococcus aureus</i> is the most common causative organism. Antimicrobial resistant strains may be associated with skin infections	- Clean the affected area and provide appropriate wound care. - Incise and drain suitable abscesses where trained personnel and appropriate facilities are available. - Prescribe antibiotics when clinically indicated according to national clinical guidelines.	Refer if: - abscess is large or deep; - surrounding cellulitis is extensive; - recurrent infection occurs or if patient fails to respond to primary treatment; - the patient is immunocompromised; or - systemic illness is present.
Urgent referral	Necrotising fasciitis	1B71 Necrotising fasciitis	- Rapidly progressive soft-tissue infection (progression within hours). - Severe pain disproportionate to clinical findings. - Swelling, blistering, skin necrosis and systemic toxicity may develop rapidly.	- Recognize as a medical emergency. - Initiate emergency management according to local protocols. - Administer empirical antimicrobial therapy where appropriate before transfer. - Arrange immediate transfer to hospital.	Immediate referral for urgent surgical assessment and hospital-based management.
Urgent referral	Cellulitis not responding to initial treatment	Use the applicable 1B70.x cellulitis code. ICD-11 has no separate stem code for treatment non-response.	- Clinical deterioration or failure to improve despite appropriate initial therapy. - Consider antimicrobial resistance, an undrained abscess or an alternative diagnosis.	- Reassess the diagnosis and treatment adherence. - Evaluate for complications or deeper infection. - Escalate care according to local protocols.	Urgent referral for further investigation, microbiological assessment and specialist management.

6.1.2 Fungal skin infections

Fungal skin infections are common at the primary health care (PHC) level and are usually diagnosed clinically. Most superficial fungal infections can be managed effectively at PHC with appropriate antifungal treatment, patient education and measures to reduce recurrence.

Table 6.2. Fungal skin infections

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Tinea (dermatophytosis)	1F28 Dermatophytosis; 1F28.0 scalp; 1F28.1 nail; 1F28.2 foot; 1F28.3 genitocrural; 1F28.4 kerion; 1F28.Y other specified (including tinea corporis when no dedicated leaf code applies)	- Annular scaly plaques with an active raised border. - Tinea pedis: scaling and fissuring between the toes or on the soles. - Tinea cruris: pruritic erythematous rash in the groin. - Tinea corporis: circular scaly plaques on the trunk or limbs.	- Diagnose clinically; confirm with microscopy where available if the diagnosis is uncertain. - Treat localized skin infection with topical antifungal agents according to national clinical guidelines. - Treat tinea capitis and extensive or nail disease with systemic antifungal therapy according to national clinical guidelines.	Refer if: - diagnosis is uncertain; - kerion is present; - infection is extensive or recurrent; - nail disease is severe or complicated; or

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
				• Tinea capitis: patchy alopecia with scalp scaling, mainly in children; inflammatory kerion may occur.	• Advise patients to keep affected areas clean and dry and to avoid sharing towels, clothing or hairbrushes.	• there is no response to appropriate treatment.
Manage at PHC level (core)		Pityriasis versicolor	1F2D.0 Pityriasis versicolor	• Multiple hypo- or hyperpigmented macules with fine scaling. • Commonly affects the trunk, neck and shoulders. • More frequent in hot and humid climates. • Usually asymptomatic or mildly pruritic.	• Treat with topical antifungal preparations according to national clinical guidelines. • Advise that hypopigmentary changes may persist after successful treatment. • Counsel on the possibility of recurrence, particularly in warm climates.	Refer if: • diagnosis is uncertain; • disease is extensive or recurrent despite treatment; or • an alternative diagnosis is suspected.
Manage at PHC level (core)		Candida intertrigo	1F23.12 Flexural or intertriginous candidosis	• Bright red, macerated plaques in skin folds. • Satellite papules or pustules are characteristic (beyond the outer margin of the rash). • Common in people with obesity, diabetes or prolonged moisture exposure.	• Keep affected areas clean and dry. • Treat with topical antifungal therapy according to national clinical guidelines. • Address predisposing factors where possible, including glycaemic control and reduction of moisture. • Advise on appropriate skin care and hygiene.	Refer if: • disease is recurrent or extensive; • underlying immunodeficiency or diabetes is suspected; or • there is no response to treatment.
Manage at PHC level (core)	Recognise and consider referral	Onychomycosis*	1F28.1 Dermatophytosis of nail	• Thickened, brittle or discoloured nails. • Subungual debris and nail plate distortion are common. • Toenails are affected more often than fingernails.	• Confirm the diagnosis where feasible before prolonged systemic treatment. • Treat selected patients according to national clinical guidelines. • Explain that treatment is prolonged; clinical improvement may take several months and a normal nail post-treatment is not guaranteed • Counsel on foot hygiene and prevention of reinfection.	-If the nail is black .

* In some health systems, onychomycosis is routinely managed at PHC level. In others, prolonged systemic treatment or diagnostic confirmation may require referral. National guidance should determine the appropriate level of care.

6.1.3 Viral skin infections

Viral skin infections are commonly encountered at the primary health care (PHC) level. Most can be recognised clinically based on their characteristic morphology and distribution. Appropriate management includes patient education, infection prevention, symptomatic care, timely antiviral therapy when indicated, and recognition of conditions requiring urgent referral.

Table 6.3. Viral skin infections

PHC management category		Disease	ICD-11 MMS code(s)*	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)		Herpes simplex	1F00.0 Herpes simplex infection of skin or mucous membrane; 1F00.00 skin; 1F00.01 labialis; 1A94 anogenital herpes simplex infection; use complication-specific codes where applicable	Typical presentation • Grouped painful vesicles on an erythematous base. • Vesicles rupture to form shallow erosions or crusts. Common sites • Lips (orolabial herpes). • Genital region (genital herpes). Important features	• Provide supportive skin care and pain relief. • Initiate antiviral therapy according to national clinical guidelines when indicated. • Counsel patients on reducing transmission and avoiding direct contact with active lesions. • Advise patients to seek care promptly if lesions involve the eyes.	Refer if: • ocular involvement is suspected; • severe primary infection occurs; • infection is disseminated; • the patient is immunocompromised; or • the diagnosis is uncertain.

PHC management category		Disease	ICD-11 MMS code(s)*	Clinical recognition	PHC management	Referral criteria
				- Recurrent episodes are common. - Primary infection may be more severe.		
Manage at PHC level (core)		Herpes zoster (shingles)	1E91 Herpes zoster; 1E91.0 without complication; 1E91.1 ophthalmic zoster; use a complication-specific subcode where applicable	Typical presentation - Painful unilateral vesicular eruption. Common sites - Thoracic or cranial dermatomes. Important features - Rash usually does not cross the midline. - Pain or tingling often precedes the rash and may persist after resolution.	- Assess as early as possible after symptom onset. - Initiate antiviral therapy according to national clinical guidelines where indicated. - Provide adequate pain management. - Keep lesions clean and covered until crusted. - Counsel on avoiding contact with susceptible individuals until lesions have crusted.	Urgently refer if: - ophthalmic involvement is suspected; - neurological complications occur; - disseminated disease develops; - the patient is immunocompromised; or - severe pain or diagnostic uncertainty is present.
Manage at PHC level (core)	Recognise and consider referral	Varicella (chickenpox)	1E90 Varicella; 1E90.0 without complication; use a complication-specific subcode where applicable	Typical presentation - Generalized pruritic vesicular rash in different stages of development. Common sites - Trunk, face and extremities. Important features - Fever commonly precedes or accompanies the rash. - Highly contagious.	- Provide symptomatic treatment according to national clinical guidelines. - Advise adequate hydration and skin care. - Recommend measures to reduce transmission until lesions have crusted. - Monitor for secondary bacterial infection and other complications.	Refer if: - severe disease develops; - complications are suspected; - the patient is pregnant, immunocompromised or a young infant, in accordance with national guidance; or - the diagnosis is uncertain.
Manage at PHC level (core)		Molluscum contagiosum	1E76 Molluscum contagiosum	Typical presentation - Small, firm, dome-shaped papules with central umbilication. Common sites - Face, trunk, genital area and extremities in children. Important features - Usually self-limiting. - Lesions may be numerous in immunocompromised individuals.	- Reassure patients that the condition is usually self-limiting. - Advise measures to reduce autoinoculation and transmission. - Treat according to national clinical guidelines where treatment is indicated. - Assess for underlying immunodeficiency if lesions are extensive or atypical.	Refer if: - lesions are extensive or atypical; - immunodeficiency is suspected; - lesions involve the eyelids with ocular symptoms; or - the diagnosis is uncertain.
Manage at PHC level (core)		Viral warts	1E80 Common warts; 1E80.1 Plantar warts; use the site- or type-specific code within 1E80–1E8Z	Typical presentation - Well-defined hyperkeratotic papules or plaques, sometimes with punctate black dots. Common sites - Hands, feet and periungual skin. Important features - Usually painless, although plantar warts may be painful. - Often resolve spontaneously.	- Explain the benign nature of the condition. - Treat according to national clinical guidelines when treatment is indicated. - Advise patients not to pick lesions and to avoid sharing personal items where appropriate.	immunodeficiency is suspected - lesions are atypical; or - malignancy cannot be excluded.

6.1.4 Parasitic skin infestations

Parasitic skin infestations are common in many primary health care (PHC) settings. Management should focus on early recognition, treatment of the patient and close contacts where appropriate, prevention of reinfestation and referral when disease is severe, recurrent or complicated.

Table 6.4. Parasitic skin infestations

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)		Scabies	1G04 Scabies; 1G04.0 Classical scabies; 1G04.1 Crusted scabies	Typical presentation • Intense itching, often worse at night. • Papules, excoriations and burrows. Common sites • Finger webs, wrists, waist, genital area and skin folds. Important features • Highly contagious. • Crusted scabies may present with thick crusts and limited itching, especially in immunocompromised people.	• Treat according to national clinical guidelines. • Treat close contacts at the same time, even if asymptomatic. • Advise washing clothing, towels and bedding, or sealing items that cannot be washed. • Explain that itching may persist after successful treatment.	Refer if: • crusted scabies is suspected; • recurrent outbreaks occur; • secondary bacterial infection is severe; • the patient is immunocompromised; or • treatment fails. MDA with oral ivermectin may be considered if community prevalence is >10%
Manage at PHC level (core)		Pediculosis capitis (head lice infestation)	1G00.0 Pediculosis capitis	Typical presentation • Scalp itching. • Live lice or nits attached to hair shafts close to the scalp. Common sites • Scalp, especially behind the ears and at the nape of the neck. Important features • Common in school-age children. • Secondary bacterial infection may occur after scratching.	• Confirm by finding live lice where possible. • Treat with topical pediculicide according to national clinical guidelines. • Repeat treatment if required by the product used. • Advise combing, avoiding shared combs or hats, and checking close contacts.	Refer if: • diagnosis is uncertain; • infestation persists after appropriate treatment; • secondary infection is significant; or • extensive dermatitis occurs.
Manage at PHC level (core)	Recognise and consider referral	Pediculosis corporis (body lice infestation)	1G00.1 Pediculosis corporis	Typical presentation • Pruritic papules and excoriations. Common sites • Areas where clothing seams contact the skin, including waist, trunk, shoulders and neck. Important features • Lice usually live in clothing seams rather than on the skin. • Associated with overcrowding and limited access to hygiene facilities.	• Prioritise hygiene and environmental measures. • Advise bathing and changing into clean clothing. • Wash or heat-dry clothing, bedding and towels where feasible. • Treat secondary bacterial infection according to national clinical guidelines. • Coordinate with public health or social services where needed.	Refer or escalate if: • severe dermatitis or secondary infection occurs; • louse-borne disease is suspected; • infestation occurs in a community or residential setting; or • social support or public health intervention is required.
Manage at PHC level (core)	Recognise and consider referral	Tungiasis	1G05 Tungiasis	Typical presentation • Painful or itchy papule with a central black dot. Common sites • Feet, toes and periungual areas. Important features • Caused by embedded female sand fleas. • Multiple lesions may cause pain, disability and secondary infection.	• Remove embedded fleas safely where trained staff and sterile equipment are available. • Clean the lesion and provide wound care. • Treat secondary bacterial infection according to national clinical guidelines. • Advise footwear use, environmental control and prevention of re-exposure.	Refer if: • lesions are multiple, deep or complicated; • secondary infection is severe; • tetanus risk requires assessment according to national guidance; or • safe removal is not feasible at PHC level.
Manage at		Cutaneous larva migrans	1F68.2 Cutaneous larva migrans	Typical presentation • Intensely itchy, raised, serpiginous track.	• Diagnose clinically when presentation is typical.	Refer if: • diagnosis is uncertain;

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
PHC level (core)				Common sites • Feet, buttocks and thighs. Important features • Track may advance slowly over days. • Often follows contact with contaminated sand or soil.	• Treat with antiparasitic therapy according to national clinical guidelines. • Provide symptomatic relief for itching. • Advise footwear, protective mats and avoidance of contaminated sand or soil.	• lesions are extensive or persistent; • secondary infection occurs; or • systemic symptoms are present.

6.2 Skin neglected tropical diseases

Skin neglected tropical diseases (skin NTDs) often present with visible skin changes and are well suited to primary recognition at primary health care (PHC) level. Early detection, basic wound care, stigma reduction, disability prevention and timely referral are essential. WHO promotes an integrated approach to skin-related NTDs requiring close collaboration between PHC and specialist teams.

Table 6.5. Skin neglected tropical diseases

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Recognise and consider referral	Leprosy	1B20 Leprosy; 1B20.0 Paucibacillary leprosy; 1B20.1 Multibacillary leprosy	Typical presentation • Hypopigmented or erythematous skin patches with loss of sensation. • Thickened peripheral nerves. Important features • May cause weakness, deformity or disability if untreated.	• Examine skin lesions and test sensation. • Start or arrange multidrug therapy according to WHO and national guidelines. • Counsel on curability, adherence and stigma reduction. • Monitor for leprosy reactions and disability.	Refer if: • nerve damage, eye involvement or disability is present; • severe reaction is suspected; • diagnosis is uncertain; or • specialist rehabilitation is needed.
Manage at PHC level (core)	Recognise and consider referral	Cutaneous leishmaniasis	1F54.1 Cutaneous leishmaniasis	Typical presentation • Chronic papule, nodule or ulcer. • Raised, indurated ulcer edge. Common sites • Exposed areas such as face, arms and legs.	• Suspect in endemic areas when an ulcer does not heal. • Provide wound care and treat secondary infection if present. • Arrange diagnostic confirmation where available. • Coordinate disease-specific treatment through the NTD programme.	Refer if: • lesion is large, multiple or on the face; • mucosal disease is suspected; • diagnosis is uncertain; or • specific antileishmanial treatment is required.
Manage at PHC level (core)	Recognise and consider referral	Post-kala-azar dermal leishmaniasis	1F54.0 Visceral leishmaniasis (includes post-kala-azar dermal leishmaniasis); add extension codes when required by national coding rules	Typical presentation • Hypopigmented macules, papules or nodules after visceral leishmaniasis. Common sites • Face, trunk and limbs. Important features • Patient may otherwise appear well.	• Ask about previous visceral leishmaniasis. • Recognize possible role in ongoing transmission. • Arrange confirmation through the NTD programme.	Refer for specialist or NTD programme assessment and treatment.
Manage at PHC level (core)	Recognise and consider referral	Buruli ulcer	1B21.20 Mycobacterium ulcerans infection	Typical presentation • Painless nodule, plaque or ulcer. • Ulcer may have undermined edges. Important features • Lesions may enlarge and cause disability if untreated.	• Identify early lesions. • Provide wound care and protect function. • Arrange confirmation where available. • Treat according to WHO and national guidelines.	Refer if: • lesion is large or deep; • joint, bone or functional impairment is suspected; • surgery or advanced wound care is required; or • diagnosis is uncertain.

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)		Yaws	1C1D Yaws; 1C1D.0 Primary yaws; 1C1D.1 Secondary yaws; 1C1D.2 Tertiary yaws	Typical presentation • Papillomatous lesion or chronic ulcer, often on the lower limbs. • Multiple papules, plaques or ulcers may occur in secondary disease. Important features • Mainly affects children in endemic rural communities.	• Recognize clinically in endemic settings. • Treat according to WHO and national guidelines. • Treat contacts and support active case-finding where part of national strategy.	Refer if: • diagnosis is uncertain; • late complications are suspected; or • treatment failure occurs.
Manage at PHC level (core)		Mycetoma	1F29 Eumycetoma; 1C43 Actinomycetoma	Typical presentation • Chronic painless swelling, usually of the foot or hand. • Draining sinuses with visible grains. Important features • May invade deep tissues and bone.	• Recognize early and document lesion extent. • Provide wound care and prevent secondary infection. • Arrange diagnostic confirmation to distinguish eumycetoma from actinomycetoma.	Refer all suspected cases for specialist diagnosis, prolonged treatment planning and assessment for surgery.
Manage at PHC level (core)		Scabies	1G04 Scabies; 1G04.0 Classical scabies; 1G04.1 Crusted scabies	See Table 6.4.	See Table 6.4.	See Table 6.4.
Manage at PHC level (core)		Chromoblastomycosis	1F24 Chromoblastomycosis	Typical presentation • Chronic warty plaques, nodules or cauliflower-like lesions. Common sites • Feet and legs. Important features • Slowly progressive after traumatic inoculation.	• Suspect in chronic verrucous lesions in endemic rural settings. • Provide wound care and prevent secondary infection. • Arrange diagnostic confirmation where possible.	• secondary cancer (squamous cell carcinoma) is suspected) • joint involvement is possible; • systemic treatment or surgery is needed; or • diagnosis is uncertain.
Manage at PHC level (core)		Sporotrichosis	1F2J Sporotrichosis; 1F2J.0 Lymphocutaneous sporotrichosis; 1F2J.1 Fixed cutaneous sporotrichosis	Typical presentation • Painless nodule or ulcer after skin trauma. • Lymphocutaneous form shows nodules tracking along lymphatic vessels.	• recognize in some endemic areas potential spread by contact with cats (eg PAHO regions).	Or the patient is immunocompromised
Manage at PHC level (core)		Onchocerciasis, skin manifestations	1F6A.1 Onchocerciasis of the skin	Typical presentation • Intense itching, dermatitis, depigmentation and subcutaneous nodules. Important features • May coexist with eye disease.	• Recognize skin signs in endemic areas. • Provide symptomatic relief for itching. • Support community-directed treatment and surveillance through the NTD programme.	Refer if: • visual symptoms occur; • severe skin disease is present; • diagnosis is uncertain; or • specialist NTD assessment is required.
Manage at PHC level (core)		Lymphatic filariasis, skin and lymphoedema manifestations	1F66.3 Lymphatic filariasis; add BD93.13 Lymphoedema due to lymphatic filariasis when the chronic manifestation is documented	Typical presentation • Chronic limb or genital swelling. • Thickened skin, skin folds, hyperkeratosis or elephantiasis. • Recurrent acute inflammatory episodes may occur.	• Teach daily washing and careful drying of affected limbs. • Treat entry lesions and secondary infection according to national guidelines. • Encourage exercise, elevation and skin care. • Support morbidity management and disability prevention.	Refer if: • genital disease or hydrocele is present; • recurrent severe infection occurs; • advanced lymphoedema needs specialist care; or • diagnosis is uncertain.
Manage at PHC level (core)		Dracunculiasis (Guinea-worm disease)	1F64 Dracunculiasis	Typical presentation • Painful blister or ulcer, usually on the lower limb. • A white, thread-like worm may emerge from	• Immediately notify the national surveillance or NTD programme and proceed to proper case containment • Clean and dress the lesion and provide pain	Report every suspected case immediately to NTD programme . Urgently refer if severe secondary

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
			the lesion. Important features • Burning pain, itching and local swelling may occur. • Contact with water may stimulate worm emergence. • Temporary disability may occur	relief according to national guidelines. • Do not pull the worm forcefully or allow it to break; arrange controlled immersion and extraction by trained personnel. • Advise the patient not to enter drinking-water sources.	infection, systemic illness or other complications are present.

6.3 Inflammatory and chronic dermatoses

Inflammatory and chronic dermatoses are common causes of consultation at the primary health care (PHC) level. Many can be recognised clinically and managed with skin care, trigger avoidance, topical treatment, patient education and planned follow-up. Referral is needed when disease is severe, persistent, atypical, complicated or associated with systemic features.

Table 6.6. Inflammatory and chronic dermatoses

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Atopic eczema	EA80 Atopic eczema; EA80.0 Infantile; EA80.1 Childhood; EA80.2 Adult atopic eczema	Typical presentation • Chronic or relapsing itchy eczema. • Dry skin, erythema, excoriations and lichenification. Common sites • Infants: face and extensor areas. • Children and adults: flexures, hands and chronic plaques. Important features • Personal or family history of atopy may be present.	• Encourage regular emollient use and avoidance of irritants. • Treat flares with topical corticosteroids of appropriate potency according to national guidelines. • Manage secondary infection if present. • Counsel patients and caregivers on chronicity, treatment adherence and safe topical corticosteroid use.	Refer if: • eczema is severe, widespread or refractory; • recurrent infection occurs; • eczema herpeticum is suspected; • growth, sleep or daily functioning is significantly affected; or • diagnosis is uncertain.
Manage at PHC level (core)	Contact dermatitis	EK00 Allergic contact dermatitis; EK02 Irritant contact dermatitis; use cause-specific subcodes where documented	Typical presentation • Localized itchy erythematous rash, sometimes with vesicles or blistering. Common sites • Hands, face, eyelids or areas exposed to irritants or allergens. Important features • Distribution often corresponds to the contact area. • Allergic contact dermatitis may have sharper borders.	• Identify and avoid the likely irritant or allergen. • Advise barrier protection, including gloves where appropriate. • Use emollients to restore the skin barrier. • Treat inflammation and itching according to national clinical guidelines.	Refer if: • dermatitis is chronic, recurrent or occupationally disabling; • patch testing is needed; • severe blistering occurs; or • diagnosis is uncertain.
Manage at PHC level (core)	Seborrhoeic dermatitis	EA81 Seborrhoeic dermatitis and related conditions; EA81.0 face; EA81.1 scalp; EA81.Z unspecified	Typical presentation • Greasy yellowish scale on an erythematous background. Common sites • Scalp, eyebrows, nasolabial folds, ears and upper trunk.	• Reassure patients that the condition is common and recurrent. • Treat scalp disease with medicated shampoo according to national guidelines. • Treat facial or body flares with appropriate topical antifungal or anti-inflammatory therapy.	Refer if: • erythrodermic or disease is severe, extensive or refractory; • diagnosis is uncertain; • secondary infection occurs; or • immunosuppression is suspected.

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
			Important features • Recurrent course is common. • Cradle cap may occur in infants.	• Advise gentle skin care and trigger management.	
Manage at PHC level (core)	Psoriasis	EA90 Psoriasis; EA90.0 Plaque psoriasis; use phenotype-specific subcodes where documented	Typical presentation • Well-demarcated erythematous plaques with silvery-white scale. Common sites • Extensor surfaces of elbows, and knees, scalp and lower back. Important features • Nail pitting or onycholysis may occur. • Ask about joint pain, stiffness or swelling.	• Manage mild localized disease with emollients and topical treatment according to national guidelines. • Counsel on chronicity, triggers and adherence. • Screen for joint symptoms and relevant comorbidities where feasible. • Avoid abrupt withdrawal of potent topical or systemic corticosteroids unless specialist advice is available.	Refer if: • erythrodermic or disease is extensive or disabling or pustular; • face, palms, soles, genitals or nails are significantly affected; • psoriatic arthritis is suspected; • treatment fails; or • systemic or phototherapy treatment may be needed.
Manage at PHC level (core)	Acne vulgaris	ED80 Acne; use ED80.Z when the subtype is unspecified	Typical presentation • Comedones, papules and pustules. • Nodules or cysts may occur in more severe disease. Common sites • Face, chest and back. Important features • Scarring and psychosocial distress may occur.	• Grade severity and treat according to national clinical guidelines. • Advise gentle skin care and avoidance of picking or squeezing lesions. • Avoid antibiotic monotherapy where national guidance advises against it. • Review response and adherence.	Refer if: • severe nodular or cystic acne is present; • scarring or significant psychosocial distress occurs; • treatment fails; or • specialist therapy may be needed.
Manage at PHC level (core)	Urticaria	EB00.0 Acute urticaria; EB00.1 Chronic urticaria; EB00.Z Spontaneous urticaria, unspecified; use EB01 for inducible urticaria	Typical presentation • Itchy wheals that appear and fade within 24 hours. • Angioedema may occur. Important features • Triggers may include infection, medicines, foods or insect stings. • Chronic urticaria lasts more than 6 weeks.	• Identify and avoid likely triggers where possible. • Treat symptoms with non-sedating H1 antihistamines according to national guidelines. • Assess for anaphylaxis when urticaria is associated with respiratory, cardiovascular or gastrointestinal symptoms.	Urgently refer if: • anaphylaxis is suspected; • airway, tongue or throat swelling occurs; • systemic symptoms are present. Refer if: • urticaria is chronic, recurrent or treatment-resistant; or • diagnosis is uncertain.
Recognise and consider referral	Rosacea	ED90.0 Rosacea	Typical presentation • Persistent central facial erythema, flushing, telangiectasia, papules or pustules. Common sites • Cheeks, nose, chin and forehead. Important features • No comedones, which helps distinguish rosacea from acne. • Eye symptoms may indicate ocular rosacea.	• Identify and reduce triggers such as sun exposure, heat, alcohol and spicy foods. • Advise gentle skin care and sun protection. • Treat papulopustular disease according to national clinical guidelines.	Refer if: • ocular symptoms occur; • disease is severe or persistent; • rhinophyma develops; • treatment fails; or • diagnosis is uncertain.
Recognise and consider referral	Lichen planus	EA91 Lichen planus; use EA91.Z when the site or type is unspecified	Typical presentation • Pruritic, violaceous, flat-topped papules. • Fine white surface lines may be present. Common sites • Wrists, ankles, lower back and oral mucosa. Important features	• Provide symptomatic relief according to national clinical guidelines. • Treat localized skin disease with appropriate topical anti-inflammatory therapy. • Examine for oral, scalp or nail involvement.	Refer if: • mucosal (severe), scalp or nail disease is present; • lesions are widespread, painful, erosive or atypical; • malignancy is suspected; or • biopsy may be required.

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
				Oral disease may appear as white lacy patches or erosions.		
Manage at PHC level (core)	Recognise and consider referral	Chronic pruritus See also Table 6.9 in Chapter 6.6	EC90 Pruritus; EC90.0 due to skin disorder; EC90.1 due to systemic disorder; EC90.6 unknown cause; EC90.Z unspecified	Typical presentation - Itching lasting more than 6 weeks. - Excoriations or lichenification may be present. Important features - May occur with dry skin, eczema, scabies or systemic disease. - Primary rash may be absent.	- Examine for common skin causes, including eczema, xerosis and scabies. - Treat the identified cause where possible. - Use emollients and symptomatic treatment according to national guidelines. - Assess for systemic symptoms where feasible.	Refer if: - generalized pruritus has no clear cause; - weight loss, jaundice, fever or systemic symptoms are present; - renal, hepatic, thyroid or haematological disease is suspected; or - symptoms persist despite initial management.
Recognise and consider referral		Hidradenitis suppurativa	ED92.0 Hidradenitis suppurativa	Typical presentation - Recurrent painful nodules, abscesses, sinus tracts or scarring. Common sites - Axillae, groin, perineum and inframammary areas. Important features - Often misdiagnosed as recurrent boils. - Substantial pain, stigma and psychosocial impact may occur.	- Recognize early disease and avoid repeated incision and drainage as the only strategy. - Provide pain relief and wound care. - Counsel on smoking cessation, weight management and psychosocial support where relevant. - Treat mild disease according to national clinical guidelines.	Refer if: - disease is moderate or severe; - sinus tracts, scarring or recurrent abscesses are present; - pain or psychosocial distress is significant; - treatment fails; or - specialist medical or surgical management is needed.

6.4 Pigmentary disorders

Pigmentary disorders are common reasons for consultation at the primary health care (PHC) level. Although many are benign, some may indicate underlying systemic disease or require specialist assessment. Accurate recognition, patient counselling and appropriate referral are important to avoid unnecessary treatment and delayed diagnosis.

Table 6.7. Pigmentary disorders

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)		Vitiligo	ED63.0 Vitiligo	Typical presentation - Well-defined depigmented macules and patches. Common sites - Face, hands, feet, elbows, knees and around body orifices. Important features - Usually asymptomatic. - Hair within lesions may also become depigmented.	- Explain that vitiligo is a chronic, non-contagious condition. - Advise sun protection for depigmented skin. - Treat localized disease according to national clinical guidelines where appropriate. - Provide psychosocial support and address stigma where needed.	Refer if: - diagnosis is uncertain; - disease is rapidly progressive or extensive; - specialist treatment is indicated; or - significant psychosocial distress is present.
Manage at PHC level (core)		Melasma	ED60.1 Melasma	Typical presentation Symmetrical light to dark brown facial patches.	- Reassure patients that the condition is benign. - Advise daily sun protection.	Refer if: - diagnosis is uncertain; - pigmentation is atypical;

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
				Common sites - Cheeks, forehead, upper lip and chin. Important features - Often associated with pregnancy, hormonal factors or sun exposure.	- Discuss that treatment is prolonged and recurrence is common. - Treat according to national clinical guidelines where appropriate.	- treatment fails; or - specialist therapies are being considered.
Manage at PHC level (core)	Recognise and consider referral	Post-inflammatory hyperpigmentation	ED60.2 Postinflammatory hypermelanosis	Typical presentation - Flat brown, grey or black macules developing after skin inflammation or injury. Important features - Common following acne, eczema, burns or skin trauma. - More common in darker skin types.	- Identify and treat the underlying inflammatory skin disease. - Advise sun protection. - Explain that pigmentation often fades gradually over time. - Avoid unnecessary or potentially harmful bleaching products.	Refer if: - diagnosis is uncertain; - pigmentation is rapidly changing; - melanoma or another serious condition cannot be excluded; or - specialist treatment is required.
Manage at PHC level (core)	Recognise and consider referral	Post-inflammatory hypopigmentation	ED63.2 Postinflammatory hypomelanosis	Typical presentation - Poorly defined pale patches following inflammatory skin disease. Important features - Normal skin texture is usually preserved. - Often improves as the underlying condition resolves.	- Treat the underlying skin disease. - Reassure patients that repigmentation is often gradual. - Advise sun protection to reduce contrast with surrounding skin.	Refer if: - diagnosis is uncertain; - lesions are progressive; - vitiligo or another diagnosis is suspected; or - there is no improvement over time.
Recognise and consider referral		Pityriasis alba	EA88.4 Pityriasis alba	Typical presentation - Ill-defined hypopigmented patches with fine scaling. Common sites - Face, especially in children. Important features - Frequently associated with dry skin or atopic eczema.	- Reassure patients and caregivers that the condition is benign. - Encourage regular emollient use. - Treat associated eczema where present. - Advise sun protection.	Refer if: - diagnosis is uncertain; - lesions are extensive or atypical; or - another pigmented disorder is suspected.
Recognise and consider referral		Freckles (ephelides)	ED61.0 Ephelides	Typical presentation - Small, sharply defined light-brown macules. Common sites - Sun-exposed areas. Important features - Become darker with sun exposure and lighter during winter.	- Reassure patients that freckles are benign. - Encourage regular sun protection. - No treatment is usually required.	Refer if: - lesions change in colour, size or shape; - diagnosis is uncertain; or - melanoma is suspected.
Recognise and consider referral		Albinism (oculocutaneous albinism)	EC23.20 Oculocutaneous albinism	Typical presentation - Generalised reduction or absence of pigmentation of the skin, hair and eyes from birth. Important features - Visual impairment, photophobia or nystagmus may be present. - Increased susceptibility to sunburn, actinic damage and skin cancer.	- Explain that albinism is an inherited, lifelong and non-contagious condition. - Advise rigorous sun protection, including protective clothing, a wide-brimmed hat, shade and broad-spectrum sunscreen according to national guidance. - Examine the skin regularly and counsel on warning signs of skin cancer. - Assess visual needs, arrange eye care and provide psychosocial support where needed.	Refer for ophthalmological assessment and diagnostic confirmation where needed. Refer if visual impairment has not been assessed, syndromic features are suspected or specialist skin surveillance is required. Urgently refer any suspicious, changing or non-healing skin lesion.

6.5 Chronic wounds, ulcers and lymphatic disorders

Chronic wounds, ulcers and lymphatic disorders require structured assessment, regular follow-up and early recognition of complications. Primary health care (PHC) providers play an important role in identifying the underlying cause, providing basic wound care, preventing infection, supporting self-care and arranging referral for complex or non-healing wounds.

Table 6.8. Chronic wounds, ulcers and lymphatic disorders

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Recognise and consider referral	Chronic leg ulcers (venous and arterial)	BD74.3 Venous leg ulcer; EF60 Ischaemic ulceration of skin (arterial ulcer); add site and severity detail by postcoordination where applicable	Typical presentation - Chronic ulcer on the lower leg. Venous features - Usually around the medial malleolus. - Shallow ulcer with irregular margins. - Oedema, varicose veins or hyperpigmented surrounding skin may be present. Arterial features - Distal ulcer on toes, foot margins or pressure points. - Punched-out appearance, necrotic base, cold pale skin or weak pulses. - Pain may worsen with limb elevation.	- Assess pulses, limb temperature, pain and signs of infection. - Use compression for venous ulcers only when arterial insufficiency has been excluded according to national guidance. - Provide regular wound cleansing and appropriate dressings. - Elevate the limb for venous ulcers and encourage mobility where appropriate. - Address risk factors, including diabetes, smoking, oedema and reduced mobility.	Refer if: - arterial insufficiency is suspected; - ulcer is deep, extensive, rapidly worsening or very painful; - bone, tendon or deep tissue is exposed; - cellulitis, sepsis or gangrene is suspected; - ulcer does not improve with appropriate care; or - malignancy is suspected.
Manage at PHC level (core)	Recognise and consider referral	Diabetic foot ulcers	BD54 Diabetic foot ulcer; add the diabetes code and relevant site/severity extension codes as required	Typical presentation - Ulcer on a weight-bearing area of the foot in a person with diabetes. Important features - May be painless because of neuropathy. - Peripheral arterial disease and infection may coexist. - High risk of osteomyelitis and amputation.	- Inspect both feet and assess sensation, pulses and signs of infection. - Offload pressure according to national guidance and available resources. - Provide wound care and maintain a moist healing environment where appropriate. - Treat infection according to national clinical guidelines. - Support glycaemic control and educate patients on daily foot inspection, footwear and early care-seeking.	Urgently refer if: - deep infection, abscess, gangrene or sepsis is suspected; - critical limb ischaemia is suspected; - bone is exposed or osteomyelitis is suspected; - ulcer is deep, extensive or rapidly worsening; or - ulcer does not improve with appropriate care.
Manage at PHC level (core)	Recognise and consider referral	Pressure ulcers (pressure injuries)	EH90 Pressure ulceration; EH90.0 grade 1; EH90.1 grade 2; EH90.2 grade 3; EH90.3 grade 4; EH90.4 suspected deep tissue damage; EH90.5 ungradable; EH90.Z unspecified	Typical presentation - Localized skin or tissue injury over a bony prominence. Common sites - Sacrum, heels, hips, buttocks and ankles. Important features - Ranges from non-blanching erythema to full-thickness tissue loss. - Common in people with reduced mobility or prolonged pressure exposure.	- Assess pressure risk and inspect skin regularly. - Reposition the patient according to risk, tolerance and local guidance. - Use pressure-reducing surfaces where available. - Optimise nutrition, hydration, continence care and pain control. - Manage stage 1–2 injuries with pressure relief, skin protection and appropriate dressings where PHC capacity allows.	Refer if: - stage 3 or 4 pressure injury is present; - necrosis, tunnelling or exposed bone is present; - infection, cellulitis or sepsis is suspected; - debridement or specialist wound care is needed; or - the wound does not improve.
Manage at PHC level (core)	Recognise and consider referral	Tropical ulcers	EA40 Tropical phagedaenic ulcer	Typical presentation - Painful, rapidly enlarging ulcer in tropical settings. - May begin after minor trauma, insect bite or scratch. - most common in children and adolescents Important features - Necrotic tissue and undermined edges may be present.	- Clean the ulcer and provide appropriate wound dressings. - Remove devitalized tissue where trained staff and safe conditions are available. - Treat secondary infection according to national clinical guidelines. - Assess nutrition and provide counselling on wound protection and early care-seeking.	Refer if: - ulcer is deep, extensive or rapidly progressive; - tendon, bone or joint is exposed; - systemic infection is suspected; - there is no improvement with initial care; or - diagnosis is uncertain.

PHC management category		Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
				Malnutrition and poor wound hygiene may contribute.		
Manage at PHC level (core)	Recognise and consider referral	Sickle-cell leg ulcers	Code the ulcer by type/site and add the applicable sickle-cell disorder code using ICD-11 cluster coding; no single universal stem code represents "sickle-cell leg ulcer".	Typical presentation - Chronic painful ulcer, commonly around the ankle. Important features - Recurrent or slow-healing ulcers in a person with sickle-cell disease. - Secondary infection and significant pain may occur.	- Provide gentle wound care and pain management. - Treat infection according to national clinical guidelines. - Coordinate care with the clinician managing sickle-cell disease. - Encourage leg elevation where appropriate and assess for vascular disease before compression.	Refer if: - ulcer is large, recurrent or non-healing; - severe pain or infection occurs; - vascular disease is suspected; - advanced wound care, grafting or specialist haematology input is needed; or - diagnosis is uncertain.
Manage at PHC level (core)	Recognise and consider referral	Lymphoedema	BD93.1 Secondary lymphoedema; use a cause-specific subcode where known (for example, BD93.13 due to lymphatic filariasis)	In some regions soil-associated lymphoedema(podoconiosis) is common	access to protective footwear	Refer if: - genital lymphoedema or hydrocele is present; - recurrent severe infections occur; - advanced lymphoedema causes disability; - specialist compression, physiotherapy or surgery is needed; or - malignancy or another underlying cause is suspected.

6.6 Other common PHC presentations

This chapter is important because it covers **common reasons for consultation** that do not fit neatly into the previous categories. The emphasis is on **recognition, initial management and appropriate referral**.

Table 6.9. Other common PHC presentations

PHC management category		Disease	ICD-11 MMS code(s)*	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)		Pruritus (without an obvious rash) See also Table 6.6 in Chapter 6.3	EC90.6 Pruritus of unknown cause or EC90.Z Pruritus, unspecified; use the confirmed underlying diagnosis when identified	Typical presentation - Generalized or localized itching with no primary skin lesions. Important features - Secondary excoriations may be present. - May indicate an underlying dermatological or systemic disorder.	- Examine the skin carefully for subtle dermatoses. - Assess for common causes, including xerosis, scabies and eczema. - Use emollients and symptomatic treatment according to national clinical guidelines. - Consider underlying systemic disease if clinically indicated.	Refer if: - itching persists despite appropriate management; - systemic symptoms are present; - significant sleep disturbance or impaired quality of life occurs; or - diagnosis is uncertain.
Manage at PHC level (core)		Xerosis (dry skin)	ED54 Xerosis cutis or asteatosis	Typical presentation - Dry, rough or flaky skin. Common sites - Lower legs, arms and hands. Important features - Common in older adults and people with atopic eczema.	- Encourage regular application of emollients. - Advise gentle cleansing and avoidance of harsh soaps or irritants. - Treat associated eczema if present. - Promote adequate hydration and skin care.	Refer if: - severe fissuring or infection develops; - underlying systemic disease is suspected; or - symptoms persist despite treatment.

PHC management category	Disease	ICD-11 MMS code(s)*	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Hyperhidrosis	EE00 Hyperhidrosis; use site- or cause-specific subcodes where documented	Typical presentation - Excessive sweating beyond physiological needs. Common sites - Palms, soles, axillae and face. Important features - May significantly affect daily activities and quality of life.	- Assess whether sweating is localized or generalized. - Exclude secondary causes such as hyperthyroidism where clinically indicated. - Advise lifestyle measures and skin care. - Treat according to national clinical guidelines where appropriate.	Refer if: - secondary hyperhidrosis is suspected; - symptoms are severe or disabling; or - specialist treatment is required.
Manage at PHC level (core)	Keloids and hypertrophic scars	EE60.0 Keloid; EE60.1 Hypertrophic scar	Typical presentation - Raised fibrous scars developing after skin injury. Important features - Keloids extend beyond the original wound margins. - Hypertrophic scars remain within the wound margins.	- Reassure patients regarding the benign nature of the condition. - Discuss treatment expectations, as recurrence is common. - Treat according to national clinical guidelines where appropriate. - Advise protection of healing wounds.	Refer if: - scars cause functional impairment; - diagnosis is uncertain; - symptoms are severe; or - specialist treatment is indicated.
Recognise and consider referral	Skin fissures	Code the underlying disorder when identified (for example, ED54 Xerosis cutis or asteatosis). ICD-11 has no single universal stem code for all skin fissures.	Typical presentation - Linear cracks in dry or thickened skin. Common sites - Heels, fingertips and palms. Important features - May be painful and become secondarily infected.	- Treat the underlying skin condition. - Use emollients and protective dressings where appropriate. - Advise reduction of friction and exposure to irritants. - Treat secondary infection according to national clinical guidelines if present.	Refer if: - fissures are deep, infected or recurrent; - underlying disease is suspected; or - healing does not occur with appropriate care.

6.7 Benign skin lesions

Benign skin lesions are common at the primary health care (PHC) level. Most require only clinical recognition, patient reassurance and observation. Treatment is indicated only when lesions cause symptoms, recurrent trauma, cosmetic concern or diagnostic uncertainty. Any lesion with atypical features or suspected malignancy should be referred for further assessment.

Table 6.10. Benign skin lesions

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Seborrhoeic keratosis	2F21.0 Seborrhoeic keratosis	Typical presentation - Well-defined, waxy or verrucous papules or plaques. Common sites - Face, trunk and upper limbs. Important features "Stuck-on" appearance. - Colour ranges from light brown to black. - Common in older adults.	- Reassure that the lesion is benign. - No treatment is usually required. - Treat only if symptomatic or repeatedly traumatized, according to national clinical guidelines.	Refer if: - diagnosis is uncertain; - rapid change in appearance occurs; - recurrent bleeding or ulceration develops; or - malignancy cannot be excluded.

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Manage at PHC level (core)	Skin tags (acrochordons)	EK71.0 Fibroepithelial polyp of skin	Typical presentation • Soft, flesh-coloured pedunculated papules. Common sites • Neck, axillae, eyelids and groin. Important features • Frequently associated with friction. • Multiple lesions may occur in people with obesity or diabetes.	• Reassure patients that lesions are benign. • No treatment is usually required• Advise weight management where clinically appropriate.	Refer if: • diagnosis is uncertain; • lesions are atypical or rapidly enlarging; • recurrent bleeding occurs; or • specialist removal is required.
Manage at PHC level (core)	Epidermoid cyst	EK70.0 Epidermoid cyst	Typical presentation • Slow-growing, firm dermal nodule. Common sites • Face, neck, trunk and back. Important features • Central punctum may be visible. • May become inflamed or secondarily infected.	• Reassure patients if the cyst is uncomplicated. • Treat secondary infection according to national clinical guidelines. • Avoid routine incision unless drainage is clinically indicated. • Counsel that recurrence may occur if the cyst wall remains.	Refer if: • diagnosis is uncertain; • recurrent infection occurs; • rapid enlargement develops; • malignancy is suspected; or • surgical excision is required.
Manage at PHC level (core)	Lipoma	2E80.0 Lipoma; use the site-specific subtype where documented	Typical presentation • Soft, mobile, painless subcutaneous mass. Common sites • Trunk, shoulders, neck and upper limbs. Important features • Usually slow growing. • Skin overlying the lesion appears normal.	• Reassure patients regarding the benign nature of the lesion. • Observe if asymptomatic. • Document size and location. • Review if symptoms or growth occur.	Refer if: • lesion is rapidly enlarging; • fixed, painful or deep; • diagnosis is uncertain; or • soft-tissue sarcoma cannot be excluded.
Manage at PHC level (core)	Melanocytic naevus (common mole)	2F20.0 Common acquired melanocytic naevus; use a site-specific subtype or 2F20.0Z when unspecified	Typical presentation • Uniformly pigmented macule or papule with regular borders. Important features • Usually stable over time. • Colour and shape are symmetrical.	• Reassure patients if the lesion is clinically benign. • Advise sun protection. • Educate patients to seek review if lesions change in size, shape, colour or symptoms.	lesions are present at birth
Recognise and consider referral	Dermatofibroma	2F23.0 Dermatofibroma	Typical presentation • Firm, small papule or nodule. Common sites • Lower legs. Important features • Characteristic "dimple sign" when compressed. • Usually asymptomatic.	• Reassure patients that the lesion is benign. • No treatment is usually required.	Refer if: • diagnosis is uncertain; • rapid growth occurs; • ulceration develops; or • malignancy cannot be excluded.
Recognise and consider referral	Pyogenic granuloma	2F26 Lobular capillary haemangioma (pyogenic granuloma)	Typical presentation • Bright red, rapidly growing friable papule. Common sites • Fingers, hands, face and lips. Important features • Bleeds easily after minor trauma.	• Control bleeding where necessary. • Keep the lesion clean and protected. • Arrange treatment according to national clinical guidelines where indicated.	Refer if: • diagnosis is uncertain; • bleeding is recurrent or difficult to control; • lesion recurs after treatment; or • malignancy cannot be excluded.

6.8 Pre-cancerous and malignant skin conditions

Pre-cancerous and malignant skin conditions may initially resemble benign skin lesions. Early recognition at the primary health care (PHC) level and prompt referral are essential to improve clinical outcomes. Any persistent, changing or non-healing lesion should be assessed carefully and referred when malignancy is suspected.

Table 6.11. Pre-cancerous and malignant skin conditions

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Recognise and consider referral	Actinic keratosis	EK90.0 Actinic keratosis	Typical presentation • Rough, scaly erythematous or skin-coloured papules or plaques. Common sites • Sun-exposed skin, especially the face, ears, scalp, forearms and hands. Important features • May represent an early precursor of cutaneous squamous cell carcinoma.	• Advise lifelong sun protection. • Educate patients about monitoring for changes. • Treat according to national clinical guidelines where PHC treatment is appropriate. • Arrange follow-up where feasible.	Refer if: • diagnosis is uncertain; • lesions are extensive or recurrent; • rapid growth, ulceration or induration occurs; or • invasive squamous cell carcinoma is suspected.
Recognise and consider referral	Bowen's disease	2E64.00 Bowen disease of skin	Typical presentation • Persistent, slowly enlarging, well-demarcated scaly or crusted erythematous plaque. Common sites • Sun-exposed skin and lower legs. Important features • Represents intraepidermal squamous cell carcinoma and may resemble eczema or psoriasis.	• Document lesion size, site, duration and change over time. • Advise sun protection. • Avoid destructive treatment before diagnostic confirmation. • Arrange specialist assessment according to national pathways.	Refer all suspected cases for diagnostic confirmation and definitive management. Urgently refer if rapid growth, bleeding, ulceration, pain or induration suggests invasive squamous cell carcinoma.
Urgent referral	Cutaneous squamous cell carcinoma (cSCC)	2C31 Cutaneous squamous cell carcinoma; use site-, subtype- and histopathology-specific coding or 2C31.Z when unspecified	Typical presentation • Hyperkeratotic plaque or nodule. • Non-healing ulcer or rapidly enlarging lesion. Common sites • Sun-exposed skin, lips and ears. Important features • May bleed or become painful. • Risk of local tissue destruction and metastasis.	• Do not delay referral. • Protect the lesion from further trauma. • Document lesion size, location and duration. • Counsel the patient regarding the need for prompt specialist assessment.	Urgent referral for histopathological confirmation and definitive management.
Recognise and consider referral	Basal cell carcinoma (BCC)	2C32 Basal cell carcinoma of skin; use site- and subtype-specific coding or 2C32.Z when unspecified	Typical presentation • Pearly papule or nodule with rolled border. • Surface telangiectasia may be visible. Important features • May ulcerate ("rodent ulcer"). • Usually slow growing but locally destructive.	• Document lesion characteristics. • Advise sun protection. • Explain the need for specialist assessment.	Refer if: • diagnosis is uncertain; • lesion is on the face or another high-risk site; • lesion is recurrent or large; or • definitive treatment is required.
Urgent referral	Cutaneous melanoma	2C30 Melanoma of skin; use site- and subtype-specific coding with extension codes as applicable	Typical presentation • Pigmented lesion with asymmetry, irregular border, colour variation, increasing diameter or evolution (ABCDE features). Important features	• Assess using the ABCDE criteria where appropriate. • Avoid destructive treatment before diagnosis. • Document lesion characteristics and history.	Urgent referral (according to national pathways) for specialist assessment and biopsy.

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
			• May bleed, itch or change rapidly. • Amelanotic melanoma may lack obvious pigmentation.	• Counsel the patient regarding the importance of urgent specialist assessment.	
Urgent referral	Kaposi sarcoma	2B57.1 Kaposi sarcoma of skin	Typical presentation • Red, purple or brown macules, plaques or nodules. Common sites • Lower limbs, face and oral mucosa. Important features • Consider underlying immunosuppression, including HIV infection, where epidemiologically appropriate.	• Assess for symptoms of systemic disease. • Arrange HIV testing and counselling according to national policy where appropriate. • Refer for confirmation and staging.	Urgent referral for specialist assessment and multidisciplinary management.

6.8 Pre-cancerous and malignant skin conditions

Purpose: To support early recognition of pre-cancerous and skin cancers and ensure timely referral.

Skin cancers are common and can be cured when detected early. Primary health care (PHC) providers play a key role in identifying suspicious lesions and ensuring appropriate referral.

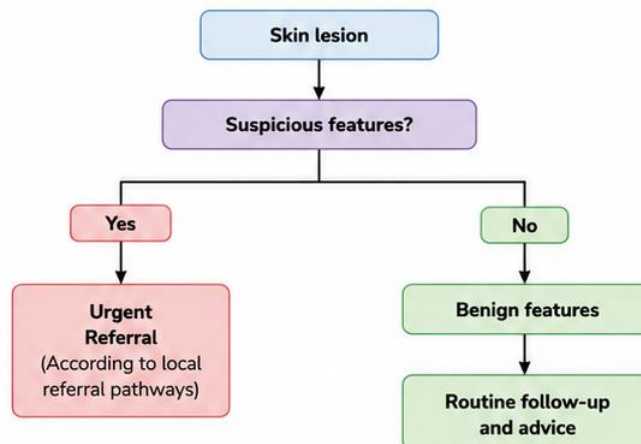
Red flags requiring urgent referral

- Rapid enlargement
- Persistent ulceration
- Spontaneous bleeding
- Pigmented lesion with change
- Non-healing lesion (> 4 weeks)
- Suspicion of melanoma
- Regional lymphadenopathy

ABCDE rule for melanoma

A	Asymmetry One half does not match the other.
B	Border irregularity Edges are uneven, notched or blurred.
C	Colour variation More than one colour or uneven spreading.
D	Diameter Usually > 6 mm, but may be smaller.
E	Evolution Change in size, shape, colour or symptoms.

Figure 6.8.1 Suspicious skin lesion pathway



When in doubt, refer. Early diagnosis saves lives.

See Section 6.7 for common benign skin lesions that may mimic skin cancer.

Disease	Typical features	Initial PHC management	Referral priority	ICD-11 MMS
Actinic keratosis	Rough, scaly erythematous or hyperkeratotic patches on sun-exposed skin.	Educate on sun protection. Consider cryotherapy or refer for treatment where available.	Routine	2D32.0 Actinic keratosis
Bowen disease (Squamous cell carcinoma in situ)	Well-demarcated, scaly or crusted erythematous plaque.	Do not excise beyond PHC scope. Refer for diagnosis and treatment.	Routine	Code as squamous cell carcinoma in situ (verify in ICD-11 browser)
Basal cell carcinoma	Pearly or waxy papule or nodule; may ulcerate or bleed.	Do not excise beyond PHC scope. Protect lesion and refer.	Prompt	Site-specific malignant neoplasm of skin (verify in ICD-11 browser)
Cutaneous squamous cell carcinoma	Scaly plaque, nodule or ulcer; may be tender; risk of invasion and metastasis.	Do not excise beyond PHC scope. Cover if ulcerated. Refer promptly.	Prompt	Site-specific malignant neoplasm of skin (verify in ICD-11 browser)
Cutaneous melanoma	Pigmented lesion with ABCDE changes; may be nodular, ulcerated or amelanotic.	Do not excise if beyond PHC scope. Refer urgently for specialist assessment.	Urgent	2C21 Malignant melanoma of skin
Kaposi sarcoma	Violaceous macules, papules or nodules; may be plaques.	Assess for HIV and associated conditions. Refer promptly.	Prompt	Site-specific malignant neoplasm (Kaposi sarcoma) (verify in ICD-11 browser)



ICD-11 MMS coding note: The ICD-11 MMS stem codes presented in this manual are intended to support standardized clinical documentation and reporting at the primary health care (PHC) level. Some conditions, including neoplasms, may require postcoordination, extension codes or multiple stem codes for complete classification. Users should follow the current WHO ICD-11 MMS release and national health information system requirements where more detailed coding is required.

6.9 Systemic, drug-related and severe skin conditions

Systemic, drug-related and severe skin conditions may be life-threatening or associated with significant morbidity. Primary health care (PHC) providers should recognise the clinical features promptly, initiate appropriate supportive care where feasible and arrange urgent referral according to national emergency pathways.

Table 6.12. Systemic, drug-related and severe skin conditions

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
Urgent referral	Stevens–Johnson syndrome (SJS)	EB13.0 Stevens-Johnson syndrome; use EH63.0 Drug-induced Stevens-Johnson syndrome when medicine causation is established	Typical presentation • Acute onset of painful erythematous or purpuric macules with blistering. • Mucosal involvement affecting the mouth, eyes or genitalia is common. Important features • Usually associated with recent medication exposure. • Patients may have fever and systemic illness.	• Stop the suspected causative medicine immediately where clinically appropriate. • Assess airway, breathing and circulation. • Provide supportive care according to local emergency protocols. • Avoid re-exposure to the suspected medicine. • Arrange urgent transfer.	Immediate emergency referral for hospital admission and specialist management.
Urgent referral	Toxic epidermal necrolysis (TEN)	EB13.1 Toxic epidermal necrolysis; use EH63.1 Drug-induced toxic epidermal necrolysis when medicine causation is established	Typical presentation • Extensive epidermal detachment with widespread blistering. • Severe mucosal involvement. Important features • Often preceded by fever and malaise. • Extensive skin loss may resemble severe burns.	• Recognise as a medical emergency. • Stop the suspected medicine immediately where appropriate. • Begin supportive care and urgent transfer according to emergency protocols.	Immediate emergency referral to a hospital with appropriate specialist care.
Urgent referral	Staphylococcal scalded skin syndrome	EA50.2 Staphylococcal scalded skin syndrome; use EH11 for neonatal staphylococcal scalded skin syndrome	Typical presentation • Fever, irritability and widespread tender erythema followed by superficial flaccid blisters and skin peeling. Important features • Usually affects neonates and young children. • Nikolsky sign may be positive; mucous membranes are typically spared.	• Recognise as a medical emergency. • Assess airway, breathing, circulation, hydration and temperature. • Protect fragile skin and prevent heat and fluid loss. • Initiate antistaphylococcal treatment according to national emergency protocols where appropriate, without delaying transfer.	Immediate emergency referral for hospital admission and systemic antibiotic and supportive management.
Manage at PHC level (core)	Drug eruptions (non-severe)	EH60 Exanthematic drug eruption	Typical presentation • Symmetrical widespread maculopapular or morbilliform rash after starting a medicine. • Itching may occur. Important features • No blistering, skin pain, mucosal involvement, facial oedema, fever or systemic illness.	• Obtain a complete medicine history and assess carefully for features of a severe cutaneous adverse reaction. • Review and stop the suspected medicine where clinically appropriate. • Provide symptomatic treatment according to national guidelines. • Document the suspected reaction, counsel on warning signs and arrange follow-up.	Refer if the diagnosis is uncertain or the eruption is extensive or progressive. Urgently refer if fever, facial oedema, skin pain, blistering, mucosal involvement or systemic symptoms develop.
Urgent referral	Drug eruption (severe cutaneous adverse reaction suspected)	EH60–EH6Z Drug eruptions; use the specific diagnosis when established (for example, EH65 DRESS, EH63 drug-induced SJS/TEN or EH64 drug-induced erythroderma)	Typical presentation • Widespread rash developing after initiation of a medicine. Important features	• Obtain a complete medicine history. • Stop the suspected medicine where clinically appropriate. • Assess severity and provide supportive care.	Urgent referral if severe cutaneous adverse reaction is suspected or systemic involvement is present.

PHC management category	Disease	ICD-11 MMS code(s)	Clinical recognition	PHC management	Referral criteria
			Fever, facial oedema, blistering, mucosal involvement or systemic symptoms suggest a severe reaction.	Document the suspected adverse reaction clearly in the patient's records.	
Urgent referral	Erythroderma (exfoliative dermatitis)	EB10 Diffuse inflammatory erythemas (includes erythroderma/exfoliative dermatitis); use EH64 when drug-induced	Typical presentation - Generalised erythema and scaling affecting most of the body surface. Important features - May occur secondary to eczema, psoriasis, medicines or malignancy. - Patients may have fever, dehydration or electrolyte imbalance.	- Assess hydration and general condition. - Maintain body temperature where possible. (Oral temperature readings may be misleadingly high, rectal temperature will be more accurate) - Apply greasy moisturizer generously to all affected skin to reduce heat loss - Arrange urgent transfer for further investigation and supportive management.	Urgent referral for hospital assessment and identification of the underlying cause.
Urgent referral	Generalised blistering disorders	ME62.6 Acute blistering skin eruption may be used provisionally when the cause is uncertain; replace with the confirmed disorder code (for example, EB40–EB49 for immunobullous disease)	.may itch	- Protect fragile skin. - Avoid unnecessary trauma to blisters. - Assess hydration and signs of infection. - Arrange urgent referral.	Urgent referral for specialist diagnosis and management.
Urgent referral	Generalised pustular eruption	EA90.40 Generalised pustular psoriasis when diagnosed; otherwise code the confirmed cause or use an appropriate provisional acute skin-eruption code	Typical presentation - Widespread sterile or inflammatory pustules on erythematous skin. Important features - Fever and systemic illness may occur. - May represent generalized pustular psoriasis or a severe drug reaction.	- Assess vital signs and general condition. - Provide supportive care according to local protocols. - Arrange urgent referral.	Immediate or urgent referral according to clinical severity.
Urgent referral	Purpura with systemic illness	ME62.1 Acute purpuric skin eruption may be used provisionally when the cause is uncertain; code the underlying cause once established	Typical presentation - Non-blanching purpuric rash associated with fever or systemic symptoms. Important features - May indicate meningococcal disease, vasculitis or other life-threatening illness.	- Assess airway, breathing and circulation. - Follow local emergency management protocols. - Arrange immediate transfer.	Immediate emergency referral.
Urgent referral	Eczema herpeticum	1F00.03 Disseminated cutaneous herpes simplex infection complicating other skin diseases	Typical presentation - Sudden onset of painful, monomorphic vesicles and erosions on areas affected by eczema. Important features - Fever and systemic symptoms may occur. - Usually caused by herpes simplex virus infection.	- Recognise as a dermatological emergency. - Arrange urgent assessment. - Initiate antiviral therapy according to national clinical guidelines where appropriate and feasible.	Immediate or urgent referral, particularly if there is ocular involvement, extensive disease or systemic illness.

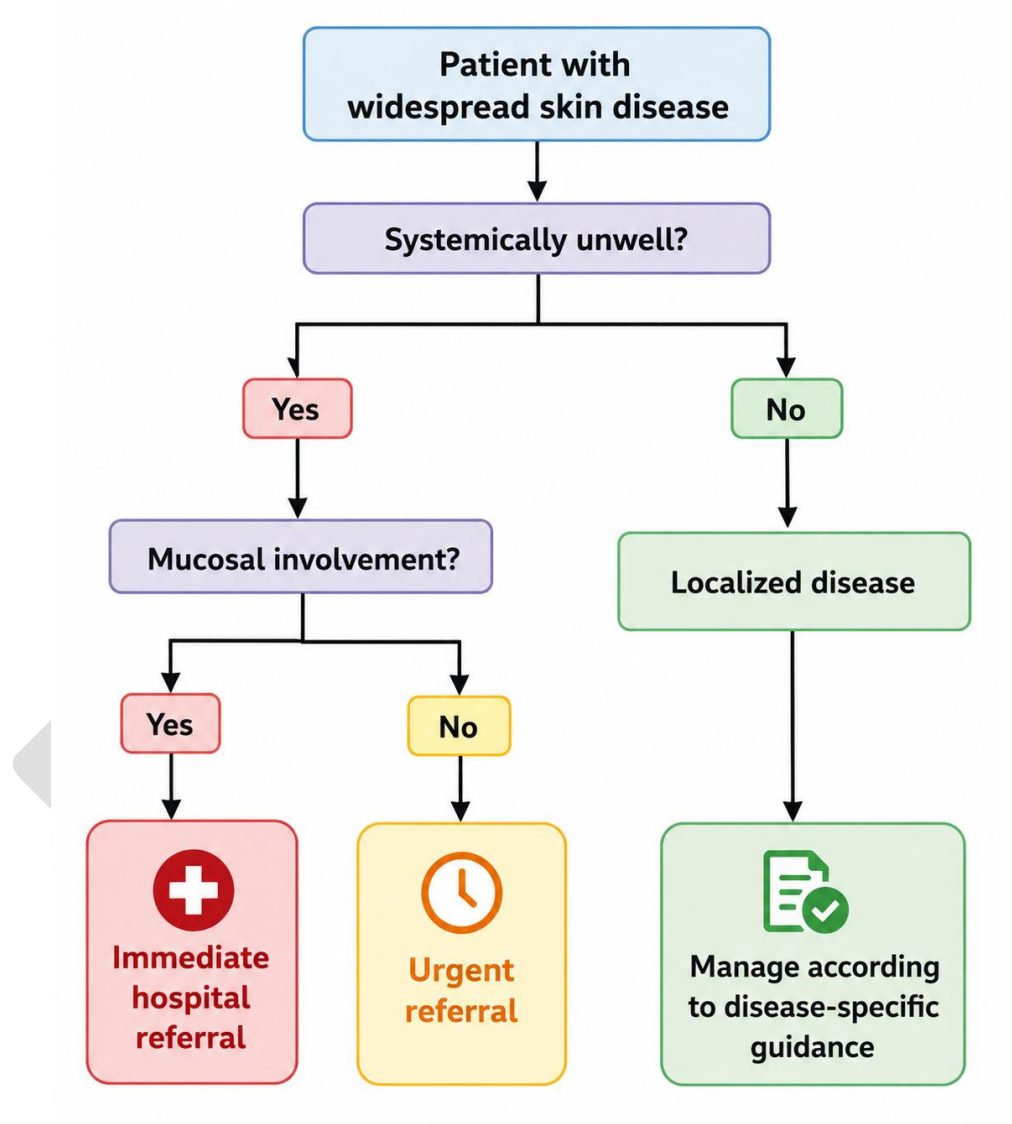


Figure 1 Recognition of Dermatological Emergencies

DRAFT

7. Methodological note

Development approach

This document was developed to support the recognition, initial management and referral of common skin diseases at the primary health care (PHC) level. It is intended as a practical clinical reference for PHC providers and programme managers and is designed to complement, not replace, national clinical guidelines.

The classification was developed using an evidence-informed approach that combined technical editing, review of authoritative reference sources and alignment with international disease classification systems. Emphasis was placed on producing clear, practical guidance that is applicable across a range of health system settings while allowing adaptation to national epidemiology, resources and policies.

Evidence sources

The classification and accompanying guidance were developed using authoritative reference sources, including:

- World Health Organization (WHO) publications and technical guidance;
- ICD-11 Mortality and Morbidity Statistics (MMS);
- International evidence-based clinical guidelines;
- Peer-reviewed scientific literature; and
- Standard dermatology reference texts where appropriate.

Recommendations were written to support primary health care practice and therefore focus on clinical recognition, initial management and appropriate referral. Where treatment recommendations vary between countries or depend on locally available medicines, the guidance refers users to national clinical guidelines. The recommendations in this manual are based primarily on WHO normative guidance, ICD-11 Mortality and Morbidity Statistics (MMS), and internationally recognized dermatology references. Users should also consult applicable national clinical guidelines and policies.

Scope of the classification

The classification includes common skin diseases that can be recognised, managed or referred at the PHC level. It is intended to support:

- early recognition of common skin diseases;
- standardised clinical assessment;
- appropriate initial management;
- timely referral of severe or complex conditions;
- standardised use of ICD-11 MMS terminology; and
- improved disease surveillance and reporting.

The classification is not intended to provide comprehensive specialist management or replace disease-specific clinical guidelines.

ICD-11 Mortality and Morbidity Statistics (MMS)

ICD-11 MMS terminology and coding have been incorporated to promote standardised disease classification, clinical documentation and health information reporting. All codes and titles in this draft were checked against the WHO ICD-11 MMS release 2026-01. ICD-11 codes should be interpreted in accordance with the current WHO release and national implementation policies. Postcoordination, cluster coding or extension codes may be required to represent site, severity, cause, histopathology or other clinical detail. Provisional symptom or eruption codes should be replaced by the confirmed diagnosis when established.

Intended use

This document is intended for use by:

- primary health care providers;
- Ministries of Health;
- programme managers;
- trainers and educators;
- public health practitioners; and
- other health professionals involved in integrated skin health services.

The guidance should always be interpreted alongside national clinical guidelines, medicine formularies and referral pathways.

Limitations

This classification does not replace clinical judgement. Individual patient management should consider clinical presentation, comorbidities, local epidemiology, available resources and national policies. Some conditions may require specialist assessment, laboratory confirmation or multidisciplinary management.

As evidence and international recommendations continue to evolve, periodic review and updating of this document are recommended.

Editorial process

The manuscript underwent technical editing to improve clarity, consistency and usability while preserving the scientific intent of the original content. Editorial revision followed WHO editorial principles, British English conventions and standardised terminology. Clinical recommendations should be reviewed and validated by subject matter experts, including dermatologists, before publication.

The document includes AI-assisted editorial support to improve language, consistency and formatting. All clinical content remains subject to expert review and final approval by the authors.

8. References

World Health Organization publications

1. World Health Organization. *International Classification of Diseases, Eleventh Revision (ICD-11) for Mortality and Morbidity Statistics (MMS)*, 2026-01 release [Internet]. Geneva: World Health Organization; 2026 [cited 2026 Jun 29]. Available from: <https://icd.who.int/browse/2026-01/mms/en>
2. World Health Organization. *ICD-11 Reference Guide* [Internet]. Geneva: World Health Organization [cited 2026 Jun 29]. Available from: <https://icdcdn.who.int/icd11referenceguide/en/html/index.html>
3. World Health Organization. *Ending the neglect to attain the Sustainable Development Goals: a strategic framework for integrated control and management of skin-related neglected tropical diseases*. Geneva: World Health Organization; 2022.
4. World Health Organization. *Ending the neglect to attain the Sustainable Development Goals: a road map for neglected tropical diseases 2021-2030*. Geneva: World Health Organization; 2020.
5. World Health Organization. *Recognizing neglected tropical diseases through changes on the skin: a training guide for front-line health workers*. Geneva: World Health Organization; 2018.
6. World Health Organization. *Guidelines for the diagnosis, treatment and prevention of leprosy*. New Delhi: WHO Regional Office for South-East Asia; 2018.
7. World Health Organization. *Consolidated guidelines on HIV prevention, testing, treatment, service delivery and monitoring: recommendations for a public health approach*. Geneva: World Health Organization; 2021.

Dermatology reference texts

8. Griffiths CEM, Barker J, Bleiker TO, Hussain W, Simpson RC, editors. *Rook's Textbook of Dermatology*. 10th ed. Hoboken (NJ): Wiley-Blackwell; 2024.
9. Kang S, Amagai M, Bruckner AL, Enk AH, Margolis DJ, McMichael AJ, Orringer JS, editors. *Fitzpatrick's Dermatology*. 9th ed. New York: McGraw-Hill Education; 2019.
10. James WD, Elston DM, Treat JR, Rosenbach M. *Andrews' Diseases of the Skin: Clinical Dermatology*. 14th ed. Philadelphia: Elsevier; 2024.
11. Dinulos JGH. *Habif's Clinical Dermatology: A Color Guide to Diagnosis and Therapy*. 7th ed. Philadelphia: Elsevier; 2019.

Primary health care

12. World Health Organization and United Nations Children's Fund (UNICEF). *Operational framework for primary health care: transforming vision into action*. Geneva: World Health Organization; 2020.

Skin cancer

13. National Comprehensive Cancer Network. *NCCN Clinical Practice Guidelines in Oncology: Basal Cell Skin Cancer*. Version 1.2026. Plymouth Meeting (PA): National Comprehensive Cancer Network; 2025 [cited 2026 Jun 29].
14. National Comprehensive Cancer Network. *NCCN Clinical Practice Guidelines in Oncology: Squamous Cell Skin Cancer*. Version 1.2026. Plymouth Meeting (PA): National Comprehensive Cancer Network; 2025 [cited 2026 Jun 29].
15. National Comprehensive Cancer Network. *NCCN Clinical Practice Guidelines in Oncology: Melanoma: Cutaneous*. Version 2.2026. Plymouth Meeting (PA): National Comprehensive Cancer Network; 2026 [cited 2026 Jun 29].

Pressure injuries and chronic wounds

16. National Pressure Injury Advisory Panel, European Pressure Ulcer Advisory Panel, Pan Pacific Pressure Injury Alliance. *Prevention and Treatment of Pressure Ulcers/Injuries: Clinical Practice Guideline* [Internet]. 4th ed. Haesler E, editor. The International Guideline; 2026 [cited 2026 Jun 29]. Available from: <https://www.internationalguideline.com/>

Drug reactions

17. Creamer D, Walsh SA, Dziewulski P, et al. U.K. guidelines for the management of Stevens-Johnson syndrome/toxic epidermal necrolysis in adults 2016. *Br J Dermatol*. 2016;174(6):1194-1227. doi:10.1111/bjd.14530.

Infectious diseases

18. Stevens DL, Bisno AL, Chambers HF, et al. Practice guidelines for the diagnosis and management of skin and soft tissue infections: 2014 update by the Infectious Diseases Society of America. *Clin Infect Dis*. 2014;59(2):e10-e52. doi:10.1093/cid/ciu296. Correction published 2015;60(9):1448. doi:10.1093/cid/civ114.
19. Nast A, Griffiths CEM, Hay R, Sterry W, Bologna JL. The 2016 International League of Dermatological Societies' revised glossary for the description of cutaneous lesions. *Br J Dermatol*. 2016;174(6):1351-1358. doi:10.1111/bjd.14419.
20. Murdoch M, Vuagnat H, Akinwuntan J, et al. Wound management in resource-limited settings: training for national and district-level health workers [Internet]. WHO Academy and ILDS Skin Health Training Hub; 2024 [cited 2026 Jun 29]. Available from: https://whoacademy.org/coursewares/course-v1:WHOAcademy-Hosted+H0065EN+H0065EN_Q1_2025 and <https://traininghub.ilds.org/courses/wound-management-in-resource-limited-settings-training-for-national-and-district-health-workers-en>

Annex 1. Primary health care approach to skin assessment

Step 1. History

Obtain a focused history, including:

- Duration and progression of the skin condition
- Initial site and pattern of spread
- **Symptoms:**
 - Itch
 - Pain
 - Fever
 - Burning sensation
- Previous episodes or recurrence
- Recent medicines, including traditional or over-the-counter products
- Allergies and previous adverse drug reactions
- Contact with affected household members or close contacts
- Recent travel or residence in endemic areas
- Occupational or environmental exposures
- Animal or insect exposure
- Personal history of skin disease
- Relevant medical conditions (e.g. diabetes, HIV or immunosuppression)
- Pregnancy status, where relevant

Step 2. Examination

Perform a systematic skin examination.

Assess the lesion

- Primary lesion type (macule, papule, plaque, vesicle, pustule, nodule or ulcer)
- Size
- Shape
- Border
- Colour
- **Surface changes:**
 - Scale
 - Crust
 - Erosion
 - Fissure
- Presence of blisters
- Presence of ulceration
- Presence of discharge

Assess the distribution

- Localized or generalized
- Symmetrical or asymmetrical
- Flexural or extensor
- Dermatomal
- Sun-exposed areas
- Skin folds
- Palms and soles
- Scalp
- Nails
- Hair
- Mucous membranes

Assess the patient

- Temperature
- General appearance
- Hydration
- Lymph node enlargement
- Peripheral nerve enlargement, where relevant (e.g. suspected leprosy)
- Limb swelling or lymphoedema
- Signs of systemic illness

Step 3. Identify red flags

Arrange urgent referral if any of the following are present:

- Fever with a widespread rash
- Rapid progression of skin lesions
- Severe pain out of proportion to clinical findings
- Extensive blistering or skin detachment
- Mucosal involvement (oral, ocular or genital)
- Purpura associated with systemic illness
- Eye involvement
- Extensive facial swelling
- Altered mental status
- Signs of sepsis or shock
- Suspected necrotising soft-tissue infection
- Extensive cellulitis
- Suspected melanoma or rapidly changing pigmented lesion
- Non-healing ulcer suspicious for malignancy

Step 4. Decide the management category

Based on the assessment, determine whether the condition should be:

Category	Action
Manage at PHC level	Provide treatment, counselling and follow-up.
Recognise and consider referral	Initiate appropriate care and refer according to local pathways where indicated.
Urgent referral	Stabilise the patient where appropriate and arrange immediate referral.

Practice points

- Examine the entire skin surface, not only the presenting lesion.
 - Assess hair, nails and mucous membranes when relevant.
 - Document lesion appearance before treatment where possible.
 - Consider underlying systemic disease when skin findings are atypical or generalized.
 - Follow national clinical guidelines and local referral pathways.
-

Annex 2. Primary Health Care Referral Checklist

Refer routinely	Urgently refer	Immediate emergency referral
Diagnostic uncertainty	Extensive or rapidly spreading skin infection	Stevens–Johnson syndrome
Treatment failure despite appropriate management	Suspected skin cancer or melanoma	Toxic epidermal necrolysis
Chronic non-healing ulcer	Ophthalmic herpes zoster	Purpura with systemic illness
Persistent inflammatory skin disease	Severe cellulitis or erysipelas	Eczema herpeticum with systemic illness
Recurrent or severe eczema	Extensive blistering disorders	Generalised blistering with mucosal involvement
Severe psoriasis requiring systemic treatment	Generalised pustular eruption	Anaphylaxis with urticaria or angioedema
Moderate or severe hidradenitis suppurativa	Diabetic foot ulcer with infection or gangrene	Necrotising fasciitis
Progressive pigmentary disorder of uncertain cause	Severe drug eruption	Septic patient with extensive skin disease
Large or recurrent chronic wounds	Advanced lymphoedema with recurrent infection	Airway compromise associated with skin disease
Lesions requiring biopsy or specialist treatment	Rapidly progressive ulceration	Any unstable patient with widespread skin disease

Clinical note: Referral urgency should always be determined using clinical judgement and national referral pathways. Patients with systemic illness, rapidly progressive disease or life-threatening skin conditions require immediate stabilisation and urgent transfer.

Annex 3. Skin Lesion Terminology

Term	Definition
Macule	A flat, circumscribed area of altered skin colour measuring less than 1 cm in diameter.
Patch	A flat area of altered skin colour measuring 1 cm or more in diameter.
Papule	A solid, elevated lesion measuring less than 1 cm in diameter.
Plaque	A raised, flat-topped lesion measuring 1 cm or more in diameter, often formed by the coalescence of papules.
Nodule	A solid, palpable lesion extending into the dermis or subcutaneous tissue, usually larger than 1 cm.
Vesicle	A small, fluid-filled blister measuring less than 1 cm in diameter.
Bulla	A fluid-filled blister measuring 1 cm or more in diameter.
Pustule	A circumscribed lesion containing purulent material.
Scale	Visible flakes of keratin that separate from the surface of the skin.
Crust	Dried serum, blood or purulent exudate on the skin surface.
Erosion	A superficial loss of the epidermis that heals without scarring.
Ulcer	A deeper loss of epidermis and dermis that may heal with scarring.
Fissure	A linear crack extending into the epidermis or dermis.
Excoriation	A superficial linear erosion caused by scratching or mechanical trauma.
Lichenification	Thickening of the skin with accentuation of normal skin markings caused by chronic rubbing or scratching.
Burrow	A thin, slightly raised serpiginous tunnel in the superficial skin, classically seen in scabies.
Telangiectasia	Permanently dilated superficial blood vessels visible on the skin surface.

Source: Morphological terminology adapted from *Rook's Textbook of Dermatology* (10th edition) and *Fitzpatrick's Dermatology* (9th edition), consistent with internationally accepted dermatological nomenclature.

Annex 4. Basic Wound Care Principles

Basic principles

Assess the wound

- Determine the duration, risk factors and cause .
 - Examine location, size, depth
 - Look for signs of infection.
- Assess pain and circulation and.
 -

Clean the wound

- Clean with clean water, sterile saline or another locally recommended solution.
- Remove visible debris where appropriate.
- Avoid unnecessary use of irritating antiseptics unless clinically indicated.

Protect surrounding skin

- Keep surrounding skin clean and dry.
- Use barrier creams or protective dressings where appropriate.

Select an appropriate dressing

- Choose a dressing that maintains a moist wound-healing environment while managing exudate.
- Change dressings according to wound condition and local protocols.

Monitor healing

- Reassess wound size, appearance and symptoms regularly.
- Document progress at each visit.

Recognise infection

- Increasing pain
- Expanding erythema
- Purulent discharge
- Malodour
- Fever or systemic illness

Refer when indicated

- Rapid deterioration
- Deep tissue involvement
- Exposed bone or tendon
- Suspected vascular insufficiency
- Failure to heal despite appropriate care
- Suspected malignancy

Practice point: Compression therapy for venous leg ulcers should only be initiated after significant arterial disease has been excluded in accordance with national clinical guidance.

DRAFT

Annex 5. Abbreviations

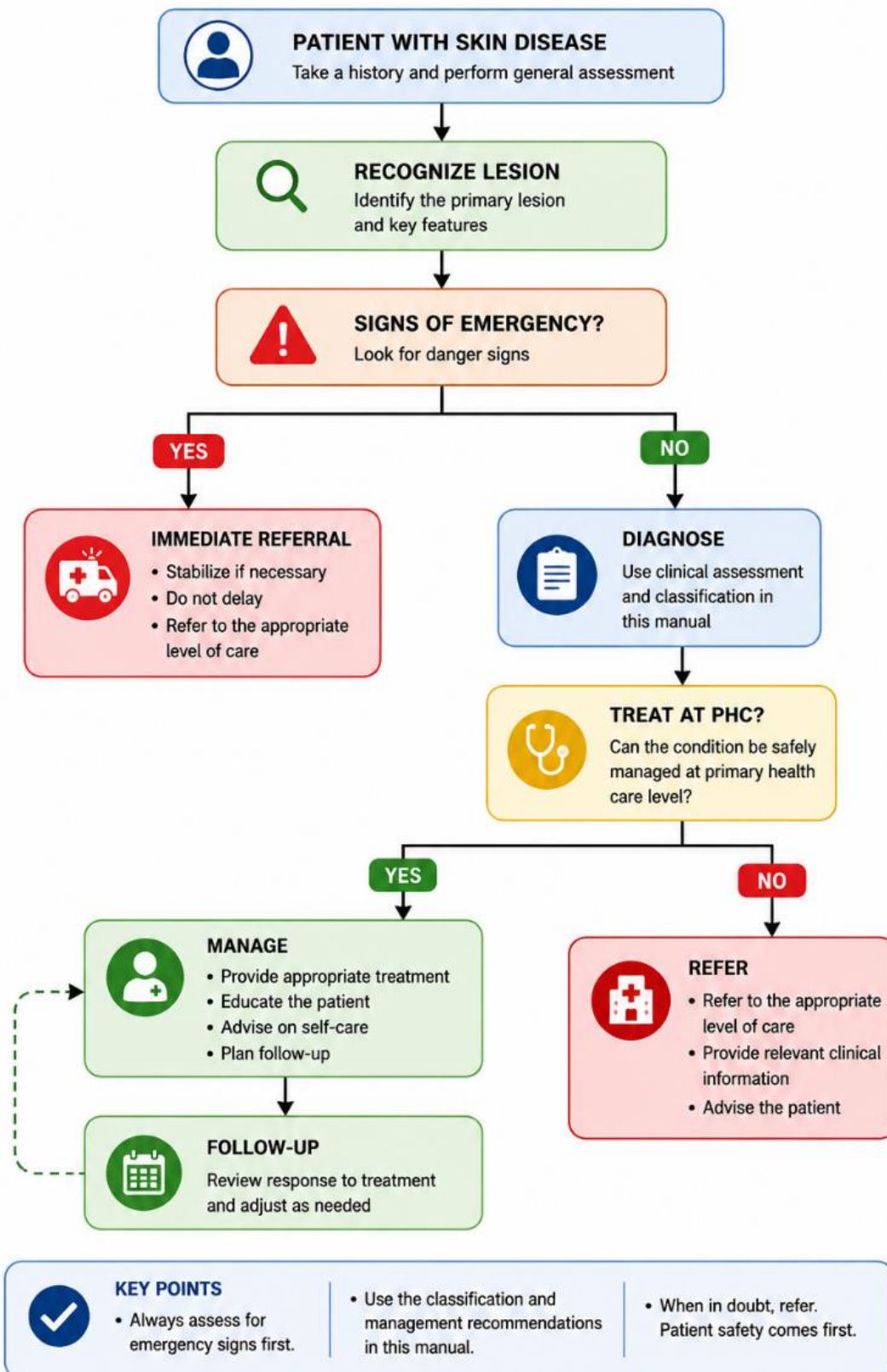
Abbreviation	Meaning
ABCDE	Asymmetry, Border irregularity, Colour variation, Diameter, Evolution
AI	Artificial intelligence
BCC	Basal cell carcinoma
cSCC	Cutaneous squamous cell carcinoma
DRESS	Drug reaction with eosinophilia and systemic symptoms
HIV	Human immunodeficiency virus
ICD-11 MMS	International Classification of Diseases 11th Revision: Mortality and Morbidity Statistics
NTD	Neglected tropical disease
PHC	Primary health care
PKDL	Post-kala-azar dermal leishmaniasis
SJS	Stevens–Johnson syndrome
TEN	Toxic epidermal necrolysis
WHO	World Health Organization

DRAFT

Annex 6. Quick reference algorithm

ANNEX 6. QUICK REFERENCE ALGORITHM

Management of Patients with Skin Diseases at Primary Health Care Level



Release Statement

This publication (Version 1.0) is released for external scientific review.

The manuscript has undergone comprehensive technical editing, editorial harmonization and publication preparation. It is intended to support structured review by subject matter experts prior to incorporation of accepted revisions in Version 1.1.

Reviewers are requested to focus primarily on scientific accuracy, clinical applicability, completeness and implementation at the primary health care level.

The Editorial Team welcomes constructive feedback that will strengthen the scientific quality and practical utility of this publication.

DRAFT